



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمة محمد فوزى ابوالحسن

المحافظة : المنوفية الحي / المدينة : اشمون

رقم البطاقة : 28504051700787 رقم التليفون : 01007077660

العمر (تاريخ الميلاد) : 05-04-1985 المهنة : ربه منزل

العنوان : مدينة نصر القاهرة

تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا أتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي :

نتائج الأبحاث : (مرفق)

التشخيص النهائي :

خطة العلاج : أبحاث أورام تصوير مقطعي محوسب مع صبغة للصدر والبطن

والحوض [11129]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمة محمد فوزى ابراهيم National ID: 28504051700787	
--	---

تقرير طبي

السيد
تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of uterine cancer for Contrast-enhanced CT scan of the chest, abdomen, and pelvis.
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



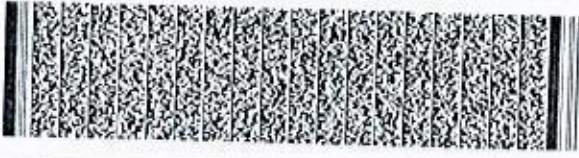
فاطمة
محمد فوزى ابوالحسن
ب ٩٨ مدخل ٤ الحى السادس
مدينة نصر اول - القاهرة

١٩٨٥/٠٤/١٠
٢٨٥٠٤٠٥١٧٠٠٧٨٧
LH5336999

٢٠٢٥/٠٨ ٢٨٥٠٤٠٥١٧٠٠٧٨٧



ربة منزل
أنثى
مسلمة
متزوجة
اسلام حسن النبوي حسن
البطاقة سارية حتى ٢٠٣٢/٠٨/١١

Patient Name: Fatma Mohamed Fawzy Abu Al Hassan	Date: 01/10/2025
Exam: CT CAP+C (CT Chest Abd Pelvis)	Age: 40 Year(s)
Medical ID: 896/2025	Pat. ID: 121833

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED:

Status post TAH-BSO and infra colic omentectomy with no available pathological data or previous films.

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Post operative status showing clear operative bed with no residual/ recurrent masses.
- Average sized liver showing homogenous parenchymal attenuation, it shows a tiny right hepatic lobe segment VI hypodense focus measuring 4.5 x 2.9 mm. no dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few tiny scattered vertebral lytic foci and tiny D10 sclerotic focus.

pinion:

- Status post TAH-BSO showing clear operative bed.
- Tiny right hepatic lobe hypodense focus for triphasic CT/ dynamic MR assessment.
- Few tiny scattered vertebral lytic foci and tiny D10 sclerotic focus for bone scan.

Sherif Wagdy
Sherif Wagdy

Name: فاطمه محمد فوزى ابو الحسن			
MR. : 6836			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	
ننشر بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 40-year-old with stage III B ovarian cancer, BRCA1-mutated, high-grade serous carcinoma. The patient underwent total abdominal hysterectomy with bilateral salpingo-oophorectomy on 6 April 2025, which revealed bilateral ovarian masses and positive omental deposits. Post-operative pathology confirmed high-grade serous carcinoma. The patient subsequently received adjuvant chemotherapy consisting of taxol and carboplatin for six cycles. After completion of chemotherapy, the patient was started on maintenance therapy with Lynparza in October 2025, continued through December 2025, and remained on stable disease as of January 2026. The patient has completed six cycles of the taxol-carboplatin regimen.			
تحريرا في ,			
التاريخ :		21-09-2026	
الوقت :			
الطبيب المعالج :		dina makram	
يعتمد ,			

Date : 01/10/2025	Patient Name: Fatma Mohamed Fawzy Abu Al Hassan
Age : 40 Year(s)	Exam : MAMMOGRAPHY BOTH
Pat. ID: 121833	Medical ID:

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: history of ovarian cancer , for routine screening .

Findings:

Right breast.

- Extremely dense breast lower the sensitivity of the mammography. (ACR D)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Extremely dense breast lower the sensitivity of the mammography. (ACR D)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

- *Right breast (BIRADS 2): breast adenosis.*
- *Left breast (BIRADS 2): breast adenosis.*

Recommendation:-

- *Right breast: for annual screening.*
- *Left breast: for annual screening.*

Best Regards,
DR. RANIA AHMED, MD
Signature Dr. Amal Hegazy
01/10/2025 12:21 PM



خطابات أورام

Nuclear Medicine Unit

Patient Name: Fatma Mohamed Fawzi

Date: 18/10/2025

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Referring physician: Prof.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- No definite active osseous metastatic bony lesions
- Abnormal heterogenous radiotracer uptake is seen at D/L spine , more advanced at mid dorsal , lumbar vertebrae , likely of benign degenerative changes .
- Bilateral shoulders , both knees , joints are seen with osteoarthritic changes , .
- Both kidneys are normally seen.

Conclusion:

- No evidence of active osseous bony lesions could be seen all over the surveyed body ,

Nuclear Medicine Unit

DR. Sherien Mahdy, MD

DR/ Mohamed Emam

Patient Name : الأستاذة / فاطمة محمد فوزي ابو الحسن	Request No. : 13004507
Patient No. : 28504051700787	Request Date : 06/04/2025
Path No. : 1191/25	Verification Date : 30/04/2025
Séx : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 40 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- The cervix showed chronic non specific inflammation & free
- The endometrium was formed of uneven tubular and rounded glands surrounded by spindled stroma and free
- 2- Right lymph nodes: Free reative nodes (0/3)
- 3- Left lymph node: Free reactive nodes (0/8)
- 4- Cytological examination revealed inflammatory cells and reactive mesothelial cells in proteinous background

* IPT:

- WT1: Positive
- P53: Mutant (Strong diffuse nuclear staining)
- ER and PR: Positive

Diagnosis:

Total abdominal hystrectomy and bilateral salpingo-oophorectomy, Infraolic omentetomy and Bilateral lymph node biopsy:

1- Bilateral ovarian high grade adenocarcinoma (serous type)

- The tumor infiltrating the omentum and serosal nodules in both uterine corpus and bladder.

- Chronic cervicitis

- Disoreded proliferative endometrium

2- Right lymph nodes: Free (0/3)

3- Left lymph nodes: Free (0/8)

4- Cytology: Mild inflammatory smears with reactive mesothelial cells in a protinaous background

* FIGO stage: III B

Signature

Dr. Rana Fathy

Dr. Marwa Mahmoud

Verified by :

Dr.Toka Medhat

Printing Date : 05/05/2025 11:37:49

Page 2 of 3

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadima - Cairo, Egypt

Patient Name : الأستاذة / فاطمة محمد فوزي ابو الحسن

Patient No. : 28504051700787

Path No. : 1191/25

Sex : Female

Age : 40 Y

Referring Doc :

Request No. : 13004507

Request Date : 06/04/2025

Verification Date : 30/04/2025

Financial Category : نفقة الدولة - اورام دار السلام

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data: A case of ovarian cancer

Nature of specimen: Total abdominal hysterectomy, Bilateral salpingo-oophorectomy, infracolic omentectomy and bilateral lymph node biopsy.

Gross Examination:

1- Total abdominal hysterectomy and Bilateral salpingo-oophorectomy measured as followed:

- Uterine corpus measured 6x6x2 cm, cervix 3.5x2.5 cm.

right ovary measured 8x6x3 cm, right tube measured 7 cm long, left ovary measured 12x11x2 cm, left tube measured 6 cm long, both ovaries were distorted, totally replaced by soft to rubbery tan mass with capsular disruption grossly, cut section was necrotic hemorrhagic.

- On opening uterine cavity measured 5x3.5cm, endometrium measured 0.2 cm thick, myometrium measured 1.8 cm thick showing trabeculations.

- Also anterior uterine serosal surface showed nodule measured 0.5x0.5 cm.

2- Right lymph nodes: Fibrofatty tissue piece measured 4x4x1 cm, cut section revealed 3 nodules up to 0.5x0.5 cm.

3- Left lymph nodes: Fibrofatty tissue piece measured 5x5x1 cm, cut section revealed 8 nodules up to 1.5x0.7 cm

4- Ascites: Two syringes 20 ml and 15 ml of hemorrhagic fluid.

Microscopic Examination:

1- Histopathological examination of both ovarian masses revealed malignant growth formed of sheets cribriform and papillae of markedly anaplastic epithelial cells having markedly pleomorphic bizarre nuclei with frequent mitosis and surrounded by wide necrosis.

- Both tubes and parametrium were free

- The omental nodules were positive for the same tumor

- The serosal nodules on surface of uterine corpus & bladder were positive for tumor

Verified by :

Dr.Toka Medhat

Printing Date : 05/05/2025 11:37:49

Page 1 of 3

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



L A B S

Collection Date: 02/07/2025
Review Date: 21/08/2025

Patient Name: الأستاذة/ فاطمة محمد أبو حسن
Patient No.: 82-32870
Accession No.: 8225077432
Age: 40Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Clinical information

Fatma Mohamed is a Cancer patient, tested for germline mutations in BRCA 1,2 for the possible applicability for targeted therapy.

Test information

Extracted DNA using QIAamp DNA Mini kit from the patient's blood sample had successfully passed appropriate libraries according to oncoReveal™ BRCA1 & BRCA2 plus CNV by Pillar bioscience and sequenced on the MiSeq platform (Illumina). The minAmp (minimum depth) was not less than X300.

- The coding regions of BRCA1 & BRCA2 genes have been investigated using NGS technology.
- BRCA1 & BRCA2 genes have been implicated in hereditary breast, ovarian, prostate and pancreatic cancer predisposition syndrome.
- For patients with BRCA1/2 mutations (actionable pathogenic or Likely pathogenic Variants), The PARP inhibitors show clear benefit (Gradishar et al., 2021)
- All identified variants are evaluated with respect to their pathogenicity and causality, and were categorized into classes (1 – 5) as recommended by ACMG guidelines, where Class (1) Pathogenic, Class (2) Likely pathogenic, Class (3) Variant of Uncertain Significance (VUS), Class (4) Likely benign, Class (5) Benign (Richards S, et al., 2015).
- Our variant classification pipeline included variant interpreter by Illumina, Clinvar and Varsome (Kopanos C. et al., 2019).
- Only pathogenic and likely pathogenic variants with actionable therapeutic application are reported.
- The test results were reviewed, interpreted and reported by our scientific and medical experts.
- The classification of variants can change over the time.

Test result

- The patient is harboring Pathogenic Variant Detected in BRCA1 gene.
- The patient is likely to benefit of PARPI.

Detected Variants

- Gene: BRCA1
- NM_007294.4:c.1224del p.Val409Ter
- exon 10
- dbSNP rs879255320
- Zygosity: Heterozygous
- Classification: Pathogenic

Interpretation

PVS1: Null variant (frameshift indel) in a gene where loss of function is a known mechanism of disease. Predicted to undergo NMD.

PS4: This variant has been reported in an individual affected with breast and/or ovarian cancer (PMID: 24249303, 26187060). This premature translational stop signal has been observed in individual(s) with hereditary breast and ovarian cancer (PMID: 24249303).

.Pl. Branch: الطبرن

Page 1 of 3

Printed by: محمود حسن ثابت 4854

AMG
ALFA MEDICAL GROUP

A MEMBER OF ALFA MEDICAL GROUP

معامل الفا

16191

[[DOCTYPE:OTHER_NGS]]



L A B S

Collection Date: 02/07/2025
Review Date: 21/08/2025

Patient Name: الاستاذة/ فاطمة محمد أبو حنين
Patient No.: 82-32870
Accession No.: 8225077432
Age: 40Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Recommendations

We recommend Follow up with the referring Physician.

Please feel free to contact NGS team at Alfa laboratories (NGS@alfalabs.com).

Test limitations

1. Intronic variants/variants in untranslated region, promoters and intronic variants are not assessed using this method.
2. Variants not detected by the assay that was performed may impact the phenotype.
3. This test is using reference databases obtained from occidental communities.
4. Cases where the genetic variant is not identified may be due to limitations in current medical knowledge or testing technology.
5. Genetic tests are largely dependent on the clinical history, medical findings and family pedigree, and therefore clinical accurate interpretation of test results may require knowing the true biological relationships in a family. Misinterpretation of results may occur if the information provided is inaccurate or incomplete.
6. The interpretation of test results is done in accordance with the clinical history, family history and other medical and laboratory data.
7. Inaccurate results in genetic testing are very rare as these tests are highly specific and accurate.

References:

1. Garrett et al.,: BRCA1/BRCA2 :CanVIG-UK Gene-Specific Guidance 2024 (<https://www.cangene-canvaruk.org/canvig-uk-guidance>).
2. Gradishar WJ, Moran MS, Abraham J, et al. NCCN Guidelines® Insights: Breast Cancer, Version 4.2021. J Natl Compr Canc Netw. 2021;19(5):484-493. Published 2021 May 1. doi:10.6004/jnccn.2021.0023
3. Kopanos C, Tsiolkas V, Kouris A, et al. VarSome: the human genomic variant search engine. Bioinformatics. 2019;35(11):1978-1980. doi:10.1093/bioinformatics/bty897
4. Richards S, Aziz N, Bale S, et al. Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology. Genet Med. 2015;17(5):405-424. doi:10.1038/gim.2015.30

Authorized and Reviewed by:

Dr. Samar Saad

Consultant, MD

Prof. Sahar AbdAtty

Head of the NGS Unit

Pt. Branch: الطبران

Page 2 of 3

Printed by: محمود حسن ثابت 4854



Report

Patient Name: فاطمة محمد فوزي

Exam Date: 14-03-2025

MRI OF THE ABDOMEN AND PELVIS WITH IV CONTRAST:-

The study was performed on a 1.5 Tesla closed magnet system.

TECHNIQUE:-

Variable pulse sequences were taken.

FINDINGS:-

- Bilateral (large on the left) adnexal complex lesions measuring 11 x 9 cm on the left & 7 x 5 cm on the right side presenting mainly soft tissue component with multiloculated cystic component & presenting 3 x 3 cm hemorrhagic cyst on the right side (hyperintense on T1 & T2).
- The soft tissue component appears hypointense on T1 & T2 & presents inhomogenous enhancement after gadolinium administration.
- No peri- or para- aortic adenopathy.
- Normal signal void flow of the major abdominal vessels.
- Normal MRI appearance of the urinary bladder and uterus.
- Moderate ascites is noted.
- The liver is of normal MRI signal pattern, average size with no focal lesions.
- No intra- or extra-hepatic biliary dilatation.
- Normal MRI signal of the spleen with no focal lesions.
- The pancreas is of normal MRI appearance.
- Normal MRI appearance of both kidneys, that show preserved parenchyma with no evidence of focal lesions, no stones or back pressure changes elicited.

OPINION:-

- Bilateral (large on the left) adnexal complex lesions with moderate ascites, picture suspicious of ovarian neoplastic process for clinical correlation & further work up.

MUCH OBLIGED

PROF, DR. MOHAMED AMIN

DR. HOSSAM FARAHAT, M.D

CHECKED BY DR:

Typed by: R.M